CONSULT FOR AFRICA
Build. Transform. Grow.

FOR PRIVATE HOSPITALS

What we do for private hospitals

Recovering what you are owed, bringing in patients, filling posts and fixing what slows the hospital down. Run by people who have run hospitals.

FROM	WHERE	WEB
Consult for Africa	Lagos and Abuja	consultforafrica.com

A basis for a conversation, not an offer of terms.

ON ONE PAGE

We take the hardest parts of running a hospital off your desk, and we stay until they work.

Free
first review of your unpaid HMO and corporate claims

Results
recovery is paid from what we collect, nothing on money we do not bring in

Operators
former hospital chief executives and clinical leaders on the ground

One team
take one service or several; most hospitals widen the work once it pays

WHAT WE DO

- **Claims and debt recovery**
  Unpaid HMO, corporate and scheme claims, recovered claim by claim.
- **Patients and referrals**
  Healthcare marketing that is measured in admissions.
- **Turnaround**
  Cash, cost and revenue control when the numbers turn.
- **Organisational audit**
  Ten domains, one report, the decisions in order.
- **Strategy and growth**
  Service lines, pricing, new sites and structuring.
- **People and leadership**
  Recruitment, locums, assessment, HR and interim leaders.
- **Governance and quality**
  Clinical governance, accreditation and board set up.
- **Systems and specialists**
  Hospital systems, dashboards and diaspora consultants.

Start with a free claims review, or a forty five minute conversation about the one thing that worries you most.

WHY US

The difference is who is standing in the hospital when the plan has to work.

Most advisers diagnose and leave. We have run hospitals, so we diagnose and then stay to deliver.

THE USUAL ADVISER	CONSULT FOR AFRICA
Writes the report, then leaves	**Writes less, then stays to deliver it**
Partners sell, juniors deliver	**Former hospital chief executives and clinicians do the work**
Frameworks from other markets	**Built for Nigerian payers, costs and staffing**
Judged on the document	**Judged on cash collected, patients seen and posts filled**
Leaves you dependent	**Hands over systems your team can run without us**

Most of what HMOs owe you is disputed, not refused. Settle the dispute and the claim gets paid.

We recover unpaid HMO, corporate and scheme claims claim by claim, with the people at the payor who decide.

Check every claim first

Against the payor's tariff, the authorisation record and the paperwork. Claims that will not pay as they stand are repaired before anyone chases them.

Reconcile with each payor

Match what you billed against what each payor says it owes, and clear the difference with their claims lead.

Follow up, escalate, settle

Retrospective authorisations agreed, missing notes resubmitted inside the window, short payments negotiated case by case.

See every claim

You see each claim's status from submission to cash, the same view we work from.

YOU NEED THIS IF

- Receivables older than ninety days
- HMOs paying less than you billed
- Nobody owns the reconciliation

The first review is free. After that we are paid from what we recover, so there is no fee on money we do not bring in.

A marketing agency that measures admissions and referrals, not impressions.

A specialist healthcare marketing team inside a firm that has run hospitals, so the campaign is built around how patients and doctors actually choose.

Position the hospital

What you are known for, who it is for and why a patient or a referring doctor should choose you over the hospital down the road.

Bring patients in

Search, social, community and corporate campaigns, written with your clinicians and checked for compliance before they run.

Build the referral base

A known list of the doctors, clinics and HMOs who send you patients, and a plan to grow it.

Prove it worked

A live view of enquiries, bookings and the revenue they became, so spend follows what converts.

YOU NEED THIS IF

- Beds or clinics running below capacity
- Marketing spend you cannot trace to patients
- Referrals concentrated in a few doctors

Monthly retainer, a fixed price project, or both. We agree the numbers it will be judged on before it starts.

When cash tightens, the hospital needs control back before it needs a strategy.

We stop the leaks, steady the cash and put a reporting rhythm in place that management can keep after we leave.

Revenue capture

Billing integrity, price realisation, payer reconciliation and the charges that never reach an invoice.

Cost discipline

Procurement, pharmacy and consumable leakage, and staff cost against revenue.

Productivity

Theatre utilisation, bed days, clinic throughput and diagnostic yield.

Cash and control

A thirteen week cash view, a monthly management pack and a board that sees the same numbers.

YOU NEED THIS IF

- Revenue rising while cash falls
- Salaries are hard to meet
- The monthly numbers arrive late or not at all

A fixed term management mandate or an embedded team against agreed targets.

ORGANISATIONAL AUDIT

Know what is actually happening in the hospital before you decide what to change.

A structured diagnostic across ten domains, from governance and finance to clinical quality and people, delivered as a report the board can act on.

Evidence, not opinion

Documents, data, site visits, case traces and confidential staff surveys.

Ten domains

Governance, strategy, finance, revenue cycle, clinical quality, operations, people, systems, facilities and patient experience.

A maturity rating

Where each domain stands today, against what good looks like for a hospital of your size.

Decisions for the board

A short list of decisions in order, with what each one costs and returns.

YOU NEED THIS IF

- A new owner, board or chief executive
- Before a raise, a merger or an expansion
- A sense that the numbers and the reality differ

A fixed price project, typically four to eight weeks from site visit to report.

Grow where the demand is, at a price the market will pay.

Service lines

Which services to add, grow or close, from demand, margin and what you are equipped to win.

Pricing and tariff

Your tariff against the market, HMO dependency, payer mix and self-pay positioning.

New sites and feasibility

Market entry, feasibility studies, business plans and financial models a lender will read.

Structuring

Joint ventures, partnerships with visiting specialists and holding company structures, alongside your counsel.

A fixed price project with a defined output, and an execution phase if you want us alongside you.

PEOPLE AND LEADERSHIP

Fill the posts, keep the good people, and know who can lead before you appoint them.

Recruitment and locums

Permanent clinical and non-clinical hires from a verified pool, and locum cover in days.

Leadership assessment

A structured assessment of every senior candidate or sitting leader, with coaching where it helps.

HR that runs

Payroll, onboarding, staff recognition and an anonymous staff pulse, set up and handed over.

Fractional leadership

An experienced chief operating officer, medical director or commercial lead, two or three days a week.

Per placement, a monthly retainer, or a fractional leader on a fixed term.

Quality and governance that protect patients and stand up to a regulator, an insurer or a buyer.

Clinical governance

Committees, escalation, incident systems, mortality and morbidity review, and clinical audit.

Accreditation readiness

Gap analysis, closure and mock survey for SafeCare, COHSASA and JCI.

Board governance

Board and committee set up, delegation of authority and director training.

Regulatory standing

Registration, licensing and inspection readiness with the state and federal regulators.

A gap analysis first, then a readiness programme against a date you set.

The right systems underneath, and the specialists you cannot hire locally.

Hospital systems

Choosing an EMR or hospital system, overseeing the rollout and digitising the paper workflows.

Dashboards

The few numbers that run the hospital, on one screen, updated without anyone building a spreadsheet.

Diaspora specialists

Senior Nigerian consultants abroad for case review, teaching, visiting lists and service set up.

Health systems

Programme design for HMOs, state schemes and development partners.

Advisory by the project, and specialists arranged session by session.

HOW WE WORK

Four ways to engage us, and every one is agreed in writing before we start.

Paid from results

Claims and debt recovery. The first review is free and the fee comes out of what we recover.

Fixed price project

An audit, a strategy, a feasibility study or a recruitment search, with a defined output and date.

Monthly retainer

Marketing, HR or management support that runs month to month against agreed numbers.

Embedded team

A turnaround or a fractional leader inside the hospital for a fixed term.

WHAT YOU CAN EXPECT

- A named lead who has done the job you are asking us to fix.
- Confidentiality: your data stays with us and your name stays out of our marketing unless you say otherwise.
- A handover, so the improvement outlasts the engagement.

HOW TO START

Three ways to start, none of which commits you to more.

01

A free claims review

Send us your list of unpaid HMO and corporate claims. We tell you what is recoverable and how long it will take.

02

A conversation

Forty five minutes with a senior partner on the one problem that worries you most. No charge.

03

An audit

A fixed price diagnostic of the whole hospital, and a short list of decisions for the board.

SPEAK TO US

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